



EVERGREEN COMMUNITY CARE INC.

PROGRAM MANAGER MANUAL

All Tasks Aligned to MCFD Standards for Staffed Children's Residential Services (SHSS)

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Applies To	Program Managers, all Evergreen Community Care Residential Programs
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MCFD Standards Alignment	Standards A–H — Standards for Staffed Children's Residential Services (SHSS), revised April 2026
Related Documents	RC Program Manual (ECC-RC-POL-2026-003); PC Manual (ECC-PC-MAN-2026-001); HR Policy Binder (ECC-HR-POL-2026-001); OHS Program Binder (ECC-OHS-POL-2026-002); QA Program Binder (ECC-QA-2026-001); Finance and Administration Manual (ECC-FA-2026-001)
Legislative Framework	CFCSA; CCALA; RCR (amended April 2026); DRIPA; Workers Compensation Act; BC Human Rights Code; Employment Standards Act; FOIPPA; Motor Vehicle Act



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PART 1 — ROLE, ACCOUNTABILITY, AND DECISION AUTHORITY

1. ROLE OF THE PROGRAM MANAGER

MCFD SHSS Standards G.4, H.1; HR Policy Binder s.2.2; Residential Care Regulation s.66–80

1.1 Position Summary

The Program Manager (PM) holds ultimate operational accountability for Evergreen Community Care's residential programs. Reporting to the Executive Director (ED), the PM is responsible for the full implementation of MCFD Standards A through H across all programs; for the employment relationship with all direct-care staff and Program Coordinators; for MCFD and licensing partner relationships; for program quality and safety outcomes; and for the integrity of all program documentation, records, and compliance systems.

The PM is not an escalation point — the PM is the accountability anchor. The Program Coordinator (PC) implements daily; the PM owns the system, the relationships, and the outcomes.

1.2 Reporting Structure

Reports To	Executive Director
Direct Reports	Program Coordinators; senior staff on programs without a PC designation
MCFD Relationship	Primary Evergreen Community Care contact for MCFD social workers, Contract Manager, and Licensing Officer at the program level
Executive Director Relationship	Reports to ED; escalates to ED any matter involving: termination; MCFD contract breach or dispute; licensing deficiency or order; media inquiry; legal proceeding; or any matter with organizational liability implications
On-Call	Designated organizational on-call executive during PM on-call periods; receives escalations from PC on-call
Authority Boundary	PM may not: sign employment offers (ED or HR authority); commit the organization to service agreements (ED authority); authorize capital expenditures above delegated limit; make public statements to media; settle any legal claim



1.3 Decision Authority Matrix

Decision Type	PM Authority	Escalate To
Accept a new MCFD placement	Full authority within licensed capacity and program fit assessment	ED if placement is outside normal program parameters or raises safety concerns for existing residents
Decline a referral	Full authority; notify MCFD in writing within 24 hours with reason	ED if declining a referral could have contract implications
Emergency discharge of a youth	Authority in consultation with MCFD social worker; notify ED same day	ED if the situation involves media, legal exposure, or complex risk
Hire direct-care staff and PCs	Authority to recommend; HR processes offer; ED countersigns offer letters	ED for any offer outside the approved compensation grid
Issue verbal and written warnings	Full authority for Steps 1–3 progressive discipline	ED + HR for Step 4 (suspension/termination)
Terminate employment	ED + HR authority required; PM prepares documentation and recommendation	ED mandatory; HR mandatory; never unilaterally
Authorize overtime	Full authority for operational necessity; document within 24 hours	ED for patterns of overtime indicating structural understaffing
Respond to licensing officer	Full authority for routine matters; acknowledge orders in writing within required timelines	ED for any Stop Work Order, Licence Suspension, or formal compliance requirement
Respond to MCFD monitoring visits	Full authority to facilitate visits, provide documents, respond to questions	ED for any finding that constitutes a material contract breach
Approve expenditures	Per Finance Manual delegated signing authority	ED for any expenditure exceeding the PM's delegated limit
Approve leave requests	Full authority per HR Policy	ED for any leave with significant operational impact
Authorize working-alone modifications	Full authority within OHS Program requirements	ED if modification creates a systemic OHS risk



1.4 Core Accountability Areas by Standard

Accountability Area	MCFD Standard + Authority
Program description, purpose, and licensing compliance	Standard A — PM ensures program always operates within its written purpose and licensed conditions
Rights of children and youth — systemic compliance	Standard B — PM owns the complaint escalation process; reviews all unresolved complaints; ensures no systemic barrier to rights exists
Personal safety — policy, incident trends, restraint oversight	Standard C — PM reviews all CI reports; conducts or supervises post-CI analysis; owns the restraint trend review; is accountable for prohibited practice elimination
Quality of care — clinical and cultural environment	Standard D — PM reviews care quality data monthly; approves ISP exceptions; leads response to any quality finding from MCFD or licensing
Service planning — ISP quality, care team leadership	Standard E — PM chairs or co-chairs care team meetings; approves ISPs before submission to MCFD; owns the transition planning quality
Physical environment — licensing and safety compliance	Standard F — PM reviews inspection reports; ensures all Priority 1 deficiencies are corrected immediately; is accountable to the Licensing Officer for facility standards
Human resources — full HR lifecycle	Standard G — PM owns all HR decisions from recruitment through separation; is accountable for training compliance; delivers or oversees reflective supervision
Organization, contracts, finance, QA, and policy	Standard H — PM is the primary author of program compliance; conducts quarterly CI trend analysis; leads the annual MCFD self-assessment; manages the MCFD contract relationship at the program level
OHS — management accountability	Workers Compensation Act s.115 — PM is accountable for OHS program implementation; reviews all incidents; supports JHSC; responds to WorkSafeBC



PART 2 — STANDARD A: PROGRAM PURPOSE, ADMISSION, AND LICENSING

2. STANDARD A — PURPOSE AND FUNCTION OF THE PROGRAM

MCFD SHSS Standard A.1; CCALA; Residential Care Regulation s.7; Contracted Service Agreement

2.1 Program Description — PM Responsibilities

Task	Standard / Authority	Timeline
Own and maintain the written Program Description — ensure it accurately reflects current practice	A.1 — written description required	Reviewed and updated annually; updated within 30 days of any material program change
Ensure the Program Description is distributed to: every admitted youth (at intake); every new staff member (at orientation); MCFD and the Licensing Officer on request; public on request	A.1 — distribution requirements	At admission and at orientation; provide to MCFD and licensing within 5 business days of request
Review the Program Description for currency and accuracy	A.1 — annual review required	Annually in July; also upon any change to the licensed program parameters, population, or staffing model
Notify MCFD Contract Manager in writing of any material change to the program before implementation	Contracted Service Agreement	Before implementation; not after
Ensure all staff receive, read, and sign the current Program Description	A.1; G.3	At orientation; upon each annual update; within 30 days of any revision
Maintain the current	H.1 — current chart shared with MCFD	Ongoing; provide to MCFD on request



organizational chart
— update within 5
business days of
any structural
change

2.2 Admission — PM Responsibilities

⚠ The Program Manager holds admission decision authority. No placement proceeds without PM confirmation. Emergency placements where the PM is unavailable require on-call supervisor authorization and PM ratification within 24 hours.

Task	Standard / Authority	Timeline
Review all available referral documentation before confirming any placement	E.1 — admission criteria; A.1	Before placement confirmed; 48 hours maximum for emergency placements
Assess program fit: the youth's needs vs. current program capacity and current resident cohort	E.1; C.1 — safety of current residents	Before placement; document the fit assessment in writing
Confirm licensed capacity before every admission	CCALA / Licensing	Before admission confirmed; PM signs the Admission Decision Record (Appendix B)
Confirm CRC is on file for all staff who will work with the new youth before the youth arrives	G.1; Criminal Records Review Act	Before admission; any gap reported to HR and ED immediately
Decline referrals that are beyond the program's capacity — notify MCFD in writing within 24 hours	A.1; E.1; Contracted Service Agreement	Within 24 hours of decision to decline
Ensure the post-admission environmental risk assessment is initiated within 24	C.1; Safety Planning	Within 24 hours of admission; PM reviews and approves within 48 hours



hours		
Confirm the preliminary ISP is in place within 48 hours of admission	E.5	Within 48 hours; PM signs or delegates signing authority explicitly
Schedule the full ISP meeting with the MCFD social worker within 14 days	E.5	Meeting scheduled within 48 hours of admission; meeting held within 14 days
Confirm Indigenous status; initiate Cultural Plan process within 30 days for every Indigenous youth	CFCSA s.71; DRIPA	Day 1 confirmation; Cultural Plan meeting within 30 days; PM accountable for completion

2.3 Licensing Relationship — PM Responsibilities

Task	Authority	Frequency / Timeline
Ensure the current facility licence is posted prominently at every facility	CCALA / Licensing	Confirm at every site visit; update within 24 hours of renewal
Ensure the facility operates within all licence conditions at all times — capacity, staffing model, population served	CCALA / Licensing	Ongoing; PM accountable
Facilitate all Licensing Officer inspections promptly and professionally	CCALA s.28	Immediately on arrival of the Licensing Officer; never delay or obstruct
Receive all licensing inspection reports; respond with a corrective action plan within the required timeline	CCALA / Licensing	Per the timeline specified in the inspection report; never miss a licensing deadline



Notify ED immediately upon receipt of any licensing order, stop-work order, or licence suspension	CCALA / Licensing	Immediately on receipt; ED leads the organizational response
Report all changes to licensed program parameters (capacity, physical layout, program type) to the Licensing Officer before implementation	CCALA / Licensing	Before implementation
Maintain the Licence Renewal Calendar and initiate renewal at minimum 90 days before expiry	CCALA / Licensing	90 days before expiry; renewal application submitted at minimum 60 days before expiry



PART 3 — STANDARD B: RIGHTS, COMPLAINTS, PRIVACY, AND MCFD RELATIONS

3. STANDARD B — RIGHTS, COMPLAINTS, PRIVACY, AND MCFD RELATIONS

MCFD SHSS Standards B.1–B.5; CFCSA s.70; Child in Care Regulation; FOIPPA; Contracted Service Agreement

3.1 Youth Rights — PM Systemic Responsibilities

The Program Coordinator manages daily rights compliance. The Program Manager is accountable for ensuring the systemic conditions that make rights compliance possible — and for escalating immediately when those conditions break down.

Task	Standard / Authority	Frequency
Ensure all staff complete Youth Rights training before their first solo shift	B.1; G.3; CFCSA s.70	At hire; verify on training matrix monthly
Include rights practice review in every supervision session with every PC	B.1; G.4	Monthly supervision; documented
Respond to any report of a rights violation as a Critical Incident — investigate personally	B.1; CFCSA s.70; C.2	Within 24 hours of report; PM leads investigation for all rights violation allegations
Review complaint logs monthly for any pattern suggesting a systemic rights compliance gap	B.2; H.2	Monthly
Ensure no staff member has ever interfered with a youth's access to the Advocate, Ombudsperson,	B.1; CFCSA s.70	Any violation: immediate suspension pending investigation; notify MCFD same day



police, or MCFD — zero tolerance		
Ensure MCFD social workers have unrestricted access to youth and to program records	B.1; Contracted Service Agreement	Ongoing; any restriction is a contract breach to be escalated to ED immediately

3.2 Complaint Resolution — PM Responsibilities (Escalation Level)

The PC handles Steps 1–3 of the complaint process. The PM receives escalations at Step 4 and is accountable for all unresolved complaints.

Step	PM Action	Timeline
Step 4 — Escalation from PC	Receive written escalation; acknowledge to complainant; initiate PM-level investigation	Within 3 business days of receipt
Step 4 — Investigation	Review all documentation; interview involved parties; determine findings	Within 10 business days of escalation
Step 4 — Written response	Provide written response with findings, reasoning, and corrective actions	Within 10 business days of escalation receipt
Step 5 — Escalation to ED	Facilitate escalation where complainant remains unsatisfied; do not obstruct	Within 5 business days of Step 4 escalation request
Complaint log review	Review all complaints received by PCs monthly; identify patterns	Monthly; report to ED quarterly
MCFD notification	Notify MCFD social worker of any complaint involving a rights violation or a matter of potential regulatory significance	Same day as identification
Licensing officer notification	Any complaint that suggests a licensing standard deficiency is reported to the Licensing Officer	As required by CCALA and licensing conditions

3.3 MCFD Relationship — PM Responsibilities

The Program Manager is Evergreen Community Care's primary operational interface with MCFD. This relationship requires consistency, transparency, and proactive communication.

Task	Authority	Frequency
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Maintain regular contact with each youth's MCFD social worker — not only when issues arise	Contracted Service Agreement; best practice	Minimum monthly contact per youth; immediate contact for any CI or significant change
Notify the MCFD social worker of all Category 1 Critical Incidents within required timelines — if SW unavailable, call PCS (1-800-663-9122)	C.2; Contracted Service Agreement	Immediately for Category 1; same day at latest
Notify MCFD of any change in program capacity, staffing structure, or service model before implementation	Contracted Service Agreement	Before implementation
Participate in MCFD care team meetings, case conferences, and monitoring visits	E.5; Contracted Service Agreement	As scheduled; never refuse or unreasonably delay a monitoring visit
Provide MCFD with all required reports, documentation, and records within contracted timelines	Contracted Service Agreement; H.1	Per Service Agreement reporting schedule; never miss a contractual reporting deadline
Maintain the MCFD Contract Manager relationship — notify of any material service issue proactively	Contracted Service Agreement	As issues arise; never allow MCFD to learn of a significant issue from a source other than Evergreen Community Care
Receive and respond to MCFD monitoring reports and recommendations	Contracted Service Agreement; H.2	Within the timeline specified in the monitoring report; never miss an MCFD deadline
Initiate and track Service Agreement renewal process — begin 6 months	Contracted Service Agreement	6 months before expiry; involve ED in all contract renewal decisions



before expiry		
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3.4 Privacy and FOIPPA — PM Responsibilities

- PM is the organizational decision-maker on all information sharing requests that the PC cannot resolve — police requests, court orders, media inquiries, and requests from unauthorized family members
- Any suspected FOIPPA breach (unauthorized disclosure, lost records, unauthorized access) is reported to the ED within 24 hours; PM leads the initial breach containment and documentation
- PM ensures all ISRs are accessible to MCFD on request; facilitates access promptly and professionally
- PM reviews FOIPPA compliance at minimum annually — including access logs, record storage, and disposal practices — as part of the annual QA review
- Any media inquiry regarding any youth or any program incident is immediately redirected to the ED — PM does not speak to media without ED authorization



PART 4 — STANDARD C: PERSONAL SAFETY, INCIDENTS, AND OVERSIGHT

4. STANDARD C — PERSONAL SAFETY: POLICY, INCIDENTS, AND OVERSIGHT

MCFD SHSS Standards C.1–C.9; Residential Care Regulation s.57–70; MCFD Critical Incident Reporting Policy; OHS Regulation

4.1 Safety Policy — PM Responsibilities

Task	Standard / Authority	Frequency
Own and maintain all written safety policies required by Standard C — restraint policy, search policy, missing youth protocol, self-harm protocol	C.1, C.3, C.6, C.7; MCFD Policy 5.13	Reviewed and updated annually; updated within 30 days of any MCFD or legislative change
Ensure written safety policies are accessible to all staff at all facilities at all times	C.1; H.1	Confirm at every site visit; update within 24 hours of any policy revision
Conduct annual workplace violence risk assessment in collaboration with the JHSC	C.1; OHS Reg. s.4.27–4.31	Annually in July; also following any serious violent incident
Conduct annual Environmental Risk Assessment for each facility	C.1; F.1	Annually in July; also on admission of any youth with a risk profile requiring modification
Review and approve all youth-specific safety plans in the ISP before implementation	C.1; E.5	At ISP development and every ISP review; PM signs or designates signing authority explicitly
Ensure all direct-care staff who may need to use	C.3; G.3	Confirm on training matrix monthly; no exceptions



physical intervention hold current NVCI certification before any solo shift		
Review all physical restraint incidents personally within 24 hours	C.3	Within 24 hours of every restraint incident
Implement threshold review when 3+ restraints involving the same youth in 30 days	C.3; best practice	Immediate — convene urgent ISP review, clinical consultation, and MCFD notification within 5 business days of threshold being reached

4.2 Critical Incident Management — PM Responsibilities

⚠ The Program Manager is personally accountable for all Critical Incident reports submitted to MCFD. The PM co-signs every CI Report before it is submitted. A CI report sent to MCFD without PM review is a compliance failure.

Incident Category	PM Required Action	Timeline
Category 1 — Same Day (death, hospitalization, abuse allegation, suicidal crisis, restraint, missing youth, police contact, school suspension)	Ensure MCFD SW or PCS (1-800-663-9122) has been notified by staff or PC; if not, make the call personally. Notify ED same day. Review draft CI Report before MCFD submission. Initiate post-CI review.	MCFD notification: within 2 hours. CI Report co-signed and submitted: within 24 hours. ED notification: same day.
Category 2 — Next Business Day (marked behaviour change, significant medication error, property damage >\$500, unauthorized overnight)	Receive PC notification; confirm CI Report completed; co-sign; submit to MCFD. Determine if ISP review is triggered.	CI Report submitted by next business day. PM co-sign required.
Allegations of abuse or prohibited practices by staff	Immediately notify ED and HR. Implement interim measures (reassignment). Notify MCFD. Ensure RCMP involved if warranted. Do not investigate without ED/HR involvement.	Immediate — PM does not delay protective action while investigating. All allegations treated as Category 1.



Death of a child or youth	Immediately call 911 (if not already done). Immediately notify ED. Notify MCFD PCS. Notify the WorkSafeBC serious incident line (1-888-621-7233). Cooperate fully with all investigations. Do not alter any records or physical evidence.	Immediate — 911 first; then ED; then MCFD PCS; then WorkSafeBC. All within the same hour.
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4.3 Post-CI Review — PM Accountability

Within 5 business days of every Category 1 Critical Incident, the PM conducts a formal review:

- Review the CI Report and all supporting documentation
- Interview all involved staff; review shift logs from the incident period
- Identify: what happened; why; what safeguards failed or were absent; what immediate actions were taken and whether they were appropriate
- Develop corrective actions with named owners and target dates; enter in the corrective action log
- Notify the JHSC if there are worker safety implications
- Initiate ISP and BSP review if the youth's care plan requires updating based on the incident
- Present findings to the MCFD social worker; submit any required follow-up documentation
- Deliver or ensure delivery of post-incident debrief to all involved staff; confirm EFAP access was offered
- Present CI trend analysis to ED quarterly — see Part 12

4.4 Quarterly Critical Incident Trend Analysis — PM Responsibility

The Program Manager conducts a formal CI trend analysis at the end of each quarter using the Quarterly CI Trend Analysis Template (Appendix C). The analysis covers:

- Total CIs by category and type for the quarter
- Year-over-year comparison
- Incidents by youth, location, time of day, shift, and staff involved — look for patterns
- Restraint incidents: number, trend, prohibited technique occurrence (target: zero)
- Missing youth incidents: number, resolution time, recurring youth
- Medication errors: number, type, resolution
- Corrective actions: open vs. closed; overdue actions escalated
- Recommendations for program or policy change

Quarterly CI trend analysis is presented to the ED and, in summary, to the MCFD Contract Manager as part of the quarterly program update.

PART 5 — STANDARD D: QUALITY OF SERVICE CLINICAL OVERSIGHT AND CULTURE

5. STANDARD D — QUALITY OF SERVICE: CLINICAL OVERSIGHT AND CULTURE

MCFD SHSS Standards D.1–D.8; Child in Care Regulation; Residential Care Regulation s.7–50

5.1 Clinical and Cultural Oversight — PM Responsibilities

The Program Manager is the clinical lead for the residential program. The PC monitors daily implementation; the PM analyses patterns, approves clinical decisions, and shapes the therapeutic culture.

Standard D Element	PM Oversight Responsibility	Frequency
D.1 — Service environment: dignity, cultural safety, gender affirmation	Review for systemic gaps monthly from QA Tool reports; respond to any pattern of concern with a team-level intervention	Monthly QA review; team supervision
D.1.13(b) — Gender-affirming care (MCFD Standard amended April 2024)	Ensure all staff have completed mandatory gender-affirming care training; treat any misgendering incident as a Code of Ethics violation requiring PM-level response	Training: monthly matrix review. Incidents: within 24 hours.
D.2 — Family involvement	Review family contact compliance quarterly; address any programme-level barrier to authorized family contact	Quarterly; also on receipt of any complaint about family access
D.3 — Maintaining relationships	Review Cultural Plans for Indigenous youth bi-annually; ensure Nation and DAA connections are substantive, not token	Bi-annually; verify in file audit
D.4, D.5 — Community inclusion and access	Review community outing logs quarterly; confirm youth are accessing community as planned in ISPs	Quarterly QA review
D.6 — Growth and development: education	Review school attendance data monthly from PC report; address systemic school engagement issues at the program level	Monthly
D.7 — Behaviour management: positive practices	Investigate any prohibited practice allegation personally; review behaviour management approach with team in reflective supervision	Any allegation: immediate. Team review: monthly supervision.



only		
D.8 — Youth voice in decision-making	Review ISPs quarterly for evidence of genuine youth input; not token signatures but documented consultation	Bi-annual file audit; quarterly ISP review

5.2 Trauma-Informed Leadership — PM Responsibility

The Program Manager models and enforces the program's trauma-informed culture. This is not a training obligation — it is a leadership obligation:

- Respond to all behaviour incidents in supervision with a 'what happened to this youth?' lens — not a blame-focused 'what did staff do wrong?' approach
- When reviewing CI Reports, ask first: was the underlying need understood? Then: was the response appropriate?
- When a youth is described as 'difficult,' 'manipulative,' or 'attention-seeking' in documentation or conversation, redirect immediately to observable behaviour language and trauma-informed understanding
- Model self-disclosure about the emotional demands of the work in team meetings — normalize secondary trauma awareness and help-seeking
- Conduct secondary traumatic stress check-in in every supervision session with every PC

5.3 Gender-Affirming Care — PM Leadership (Standard G.4.3(c), April 2024)

▲ MCFD Standard G.4.3(c) (amended April 2024) requires that supervision explicitly address gender-affirming care practice. This means every supervision session with every PC must include a discussion of how gender-affirming care is being practiced on the team. This is not optional.

- Confirm in every PM-to-PC supervision that all staff are using correct names and pronouns for every youth consistently
- Confirm that any misgendering incident has been addressed as a Code of Ethics violation — not minimized or treated informally
- Confirm that the physical environment of the program supports gender identity expression — affirming clothing, personal care items, bedroom/bathroom arrangements
- Confirm that the youth's gender-affirming care needs are documented in their ISP and reviewed at each ISP review
- Ensure PM-level review of the program's gender-affirming care environment is documented in every supervision record

PART 6 — STANDARD E: SERVICE PLANNING, ISP QUALITY, AND TRANSITIONS

6. STANDARD E — SERVICE PLANNING: ISP QUALITY, CARE TEAMS, AND TRANSITIONS

MCFD SHSS Standards E.1–E.7; CFCSA s.70(1)(b); MCFD Child Welfare Policy Chapter 4

6.1 ISP Quality and Approval — PM Responsibilities

▲ The Program Manager approves every ISP and BSP before they are shared with or submitted to MCFD. An ISP submitted without PM approval is a compliance failure. The PM is accountable for the quality of every ISP across the program.

Task	Standard / Authority	Timeline
Approve every ISP at development — review for: genuine youth voice; measurable goals; comprehensive domains; cultural plan for Indigenous youth; safety plan adequacy	E.5	Before MCFD submission; within 14 days of admission for initial ISP
Approve every ISP at quarterly review — confirm review conducted on schedule; goals updated; youth's input documented genuinely	E.5	At each quarterly review; PM signs ISP review record
Trigger emergency ISP reviews within 5 business days of any Category 1 CI	E.5; C.2	Within 5 business days; PM chairs or designates chair for emergency review
Lead or co-chair care team meetings for complex or high-risk youth	E.5	As clinically indicated; PM leads all multi-agency case conferences
Ensure Cultural Plans for all Indigenous youth are developed	CFCSA s.71; DRIPA	Track on admission calendar; verify in bi-annual file audit



within 30 days of admission and updated at each ISP review		
Ensure every transition plan begins at minimum 60 days before anticipated discharge	E.6	Track on care plan calendar; review monthly with PC
Review transition plans for all youth approaching discharge — approve before MCFD submission	E.6	Within the 60-day transition planning window
Receive and review all discharge summaries; approve before submission to MCFD and receiving provider	E.6	Within 5 business days of discharge
Review ISP quality in bi-annual file audit — assess: goal quality; youth voice; cultural planning; safety planning; documentation completeness	E.5; E.7; H.2	Bi-annually (January and July); see Appendix D

6.2 Placement Stability and Unplanned Discharges — PM Responsibilities

Situation	PM Action	Timeline
Placement showing early signs of instability (3+ incidents, frequent absences, escalating behaviour)	Convene care team review; increase supervision and clinical consultation; notify MCFD proactively	Within 48 hours of identifying instability risk; do not wait for crisis
Youth at imminent risk of unplanned	Convene emergency care team meeting; explore all alternatives with MCFD; explore	Immediately; within 48 hours maximum



discharge	enhanced staffing, clinical consultation, family involvement, and cultural support	
Unplanned discharge cannot be avoided	Notify MCFD; ensure youth is given maximum notice and choice; facilitate warm handover; complete full discharge summary	Notify MCFD immediately. Discharge summary: within 5 business days.
Unplanned placements ending within 90 days of admission	Conduct a formal case review with MCFD social worker; identify what Evergreen Community Care could have done differently; document learnings	Within 30 days of discharge; submit findings to MCFD and ED



PART 7 — STANDARD F: SHELTER, EQUIPMENT, AND PHYSICAL ENVIRONMENT

7. STANDARD F — SHELTER, EQUIPMENT, AND PHYSICAL ENVIRONMENT

MCFD SHSS Standards F.1–F.4; Residential Care Regulation s.22–56; BC Building Code; BC Fire Code; CCALA

7.1 Physical Environment — PM Responsibilities

Task	Standard / Authority	Frequency
Review all monthly facility inspection reports (Appendix C) from PCs — sign off; identify any systemic or recurring deficiencies	F.1–F.4; OHS Reg. s.3.5	Within 5 business days of receipt; PM signs off on all inspection reports
Ensure all Priority 1 deficiencies are corrected immediately — verify correction in writing	F.4; OHS Reg.	Within 24 hours of identification; never wait for scheduled inspection to address immediate hazards
Conduct a personal site visit to each facility at minimum monthly — not only for inspections; for presence and relationship	F.1; H.1; best practice	Monthly minimum; more frequently during periods of operational challenge
Review Environmental Risk Assessment for each facility annually and update following any CI revealing an environmental hazard	C.1; F.1	Annually in July; also within 5 business days of any relevant CI
Ensure facility licence conditions are met at all times	CCALA / Licensing	Ongoing; any lapse is a licensing breach to be addressed and reported



— capacity, staffing, physical standards		immediately
Maintain the vehicle records, inspection logs, and insurance confirmation for all program vehicles	H.1; Motor Vehicle Act	Monthly vehicle log review; insurance confirmation annually; defects reported immediately
Verify hot water temperature compliance (maximum 49°C at all taps) is monitored monthly at all facilities	F.1; RCR April 2026	Monthly inspection; also on admission of any youth with burn risk
Ensure all fire evacuation drills are completed on schedule — review documentation	F.4; BC Fire Code	Quarterly (announced); annually (unannounced night/weekend); PM reviews all drill records
Track all facilities maintenance budgets and expenditures — approve any unbudgeted repair above the delegated limit	Finance Manual; H.1	Ongoing; monthly budget review



PART 8 — STANDARD G: HUMAN RESOURCES

8. STANDARD G — HUMAN RESOURCES: HIRING, TRAINING, SUPERVISION, AND PERFORMANCE

MCFD SHSS Standards G.1–G.4; Criminal Records Review Act; BC Employment Standards Act; BC Human Rights Code; HR Policy Binder (ECC-HR-POL-2026-001)

8.1 Recruitment and Hiring — PM Responsibilities (Standard G.1)

Task	Standard / Authority	Requirement
Approve all job postings for program-level positions before posting	G.1; HR Policy s.3.1	Before posting; PM ensures posting reflects current needs and job description
Conduct or participate in all hiring interviews for PC and senior direct-care positions	G.1 — personal interview required	For all permanent direct-care and PC positions; document participation in hiring record
Review all reference checks before any hiring decision is made	G.1 — reference documentation required	Before offer; PM reviews all references; any concern raised stops the process
Confirm CRC Vulnerable Sector clearance is received and documented before any new hire works with youth	G.1; Criminal Records Review Act	Absolute requirement; PM confirms with HR before first shift is scheduled; no exceptions
Ensure all job descriptions are current and approved before hiring	G.1 — written job descriptions required	Reviewed annually; PM approves any changes to the job description
Document the hiring decision rationale — including youth input on desired staff qualities	G.1 — documentation of hiring reasons; youth input	After every hiring decision; documentation retained in HR record



Ensure all personnel file contents are complete at hire — all 14 required items per Standard G.1	G.1 — required personnel records	Verify with HR within 30 days of hire; PM signs off on file completeness
Approve all offers of employment — ensure offer letters are processed through HR/ED signing authority	HR Policy — PM does not sign offers	Approve the offer recommendation; offers signed by HR/ED

8.2 Staffing Ratios and Scheduling — PM Oversight (Standard G.2)

Task	Standard / Authority	Frequency
Review and approve all shift schedules prepared by PCs — confirm ratios meet licensing requirements	G.2; RCR s.66	Weekly schedule review before publication; any ratio concern addressed before the schedule takes effect
Maintain the written emergency additional staff plan — update at minimum annually and whenever staff turnover affects the casual pool	G.2 — written plan required	Annually; also on any significant staffing change
Review all below-minimum staffing events — investigate cause, confirm corrective action, document	G.2; C.1	Within 48 hours of any below-minimum staffing event reported by PC
Monitor cumulative overtime by individual staff member — manage for fatigue and retention risks	HR Policy; OHS best practice — fatigue management	Monthly payroll review; any staff member exceeding threshold flagged for discussion
Review and	G.2	Before implementation of



approve any permanent schedule change — confirm it meets G.2 requirements and does not reduce coverage below minimum		any structural schedule change
Ensure the on-call supervisor schedule is published one week in advance and always covered	G.2; C.1	Weekly; PM is the escalation point if on-call coverage cannot be arranged

8.3 Training Compliance — PM Accountability (Standard G.3)

The Program Manager is accountable for training compliance across the entire program. The PC monitors monthly; the PM holds the PM accountable and receives the monthly reports.

Task	Standard / Authority	Frequency
Receive and review monthly training compliance reports from all PCs — identify any lapse requiring immediate action	G.3 — training records maintained	Monthly — by the 15th; PM reviews by the 20th
Ensure any staff member with a lapsed mandatory training is removed from duties requiring that training immediately — confirm with PC	G.3; HR Policy	Within 24 hours of identifying any lapse; PM confirms action taken
Approve all training schedules and ensure mandatory training is booked ahead of deadlines — not on deadline	G.3	Ongoing; review upcoming expiries in monthly training report
Ensure gender-	G.3.2(b) — MANDATORY since April 2024	Monthly review; PM



affirming care training is completed by all staff within 60 days of hire and renewed annually — this is a MANDATORY Standard G.3.2(b) requirement since April 2024		personally confirms compliance; no lapse acceptable
Ensure Indigenous Cultural Safety and Humility training is completed by all staff within 60 days and renewed annually	G.3; DRIPA	Monthly review
Present training compliance data to MCFD at monitoring visits and to the Licensing Officer on request	G.3; H.1	On request; maintain current data
Approve the annual training budget — ensure all mandatory training is fully funded	H.1 — budget; G.3	Annually in budget planning; PM advocates for adequate training resources
Approve professional development requests — \$500 minimum per full-time employee annually	HR Policy s.5.3	By March 31 for planning; PM approves individual requests throughout the year

8.4 Supervision — PM Delivery and Oversight (Standard G.4)

The Program Manager provides direct reflective supervision to Program Coordinators. The PM also sets the supervision culture for the entire program through how they model and enforce supervision expectations.

Task	Standard / Authority	Frequency
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Provide individual reflective supervision to every PC — minimum monthly; bi-weekly for new PCs and PCs on PIPs	G.4 — regular supervision required	Monthly minimum; bi-weekly for new PCs (first 90 days) and PIPs
Address gender-affirming care practice in every supervision session with every PC (Standard G.4.3(c), amended April 2024)	G.4.3(c) — MANDATORY since April 2024	Every session; documented in supervision record
Review and monitor PC supervision records for all direct-care staff — ensure monthly supervision is occurring	G.4	Monthly; request PC supervision records; address any gap
Conduct 30-day, 90-day, and 180-day probationary reviews for all PCs personally	G.4; HR Policy s.4.3	Per schedule; written documentation; PM and employee both sign
Conduct annual written performance evaluations for all PCs	G.4 — regular written evaluations required	Annually; coordinate timing with HR
Conduct probationary check-ins for any direct-care staff where the PC requires PM involvement	G.4; HR Policy	As needed; PM attends PC-led 90-day reviews where the staff member is borderline
Issue Performance Improvement Plans for PCs — in collaboration with HR	G.4; HR Policy s.13	On identification of performance below standard; PM drafts with HR; ED informed
Apply progressive discipline for PCs	G.4; HR Policy s.13–14	Consistently; always documented; always with



— Steps 1 through 3 with HR involvement; Step 4 to ED/HR		HR awareness
Approve and review exit interviews for all departing staff	G.4; HR Policy — exit interview required	Within 5 business days of departure; PM reviews for themes in supervision practices
Facilitate all-staff meetings minimum quarterly — PM leads organizational updates	G.4 — communication mechanisms	Quarterly; PM presence required; agenda includes organizational direction, policy updates, recognition
Respond to all staff grievances at Step 4 (PM investigation level) within 10 business days	HR Policy s.17 — grievance procedure	Within 10 business days of receiving Step 4 escalation; PM investigates and responds in writing

8.5 Progressive Discipline and Termination — PM Responsibilities

Discipline Level	PM Role	Authority / Process
Step 1 — Verbal warning (documented)	PM delivers verbal warnings for PCs; reviews PC delivery of verbal warnings for direct-care staff	PM authority; PM ensures documentation
Step 2 — Written warning	PM issues written warnings for PCs; reviews and co-signs PC-issued warnings for direct-care staff	PM authority with HR awareness; HR reviews draft before delivery
Step 3 — Final written warning / PIP	PM issues for PCs; reviews and approves PC-issued for direct-care staff; PIP developed in consultation with HR	PM authority; HR must review and approve the PIP before delivery
Step 4 — Suspension or termination	PM prepares recommendation and documentation; presents to ED and HR; does NOT implement unilaterally	ED + HR authority; PM presents; ED decides; HR administers
Summary dismissal (abuse, prohibited practices, CRC disclosure, serious misconduct)	PM implements immediate suspension with pay pending investigation; notifies ED and HR immediately; does not terminate without ED/HR involvement	PM authority for interim suspension; ED + HR for termination



PART 9 — STANDARD H: ORGANIZATION, CONTRACTS, FINANCE, AND QA

9. STANDARD H — ORGANIZATION, CONTRACTS, FINANCE, AND QUALITY ASSURANCE

MCFD SHSS Standards H.1–H.2; Contracted Service Agreement; QA Program Binder (ECC-QA-2026-001); Finance and Administration Manual

9.1 Administration and Policy — PM Responsibilities (Standard H.1)

Task	Standard / Authority	Frequency
Own all written policies and procedures for the program — ensure they reflect current MCFD Standards and legislation	H.1 — current written policy required	Reviewed annually in July; updated within 30 days of any relevant change
Communicate all policy changes to all affected staff — obtain signed acknowledgement within 30 days	H.1 — communication of changes	Within 30 days of any policy change; document receipt
Ensure all program records are stored securely and retained per the retention schedule	H.1; FOIPPA	Ongoing; verify at bi-annual file audit
Maintain the organizational chart current — share with MCFD following any structural change	H.1	Within 5 business days of any structural change
Manage the MCFD Service Agreement — track all obligations, deliverables, and reporting deadlines	Contracted Service Agreement	Ongoing; PM maintains the Contract Tracking Log (Appendix G)
Submit all required MCFD reports	Contracted Service Agreement	Per Service Agreement schedule; PM never misses



within contracted timelines — PM is accountable for all reporting deadlines		an MCFD reporting deadline
Review all insurance policies annually — confirm adequate coverage; notify ED of any gap	H.1 — insurance reviewed annually	Annually; report to ED
Confirm all vehicles used to transport youth are insured, registered, inspected, and maintained	H.1; Motor Vehicle Act	Monthly vehicle log review; annual comprehensive review
Review and co-sign all monthly financial statements — confirm program expenditures are within budget	H.1; Finance Manual	Monthly; PM signs financial statement review; any variance >10% explained in writing
Prepare the annual program budget — submit to ED by the budget deadline	H.1; Finance Manual	Annually; per organizational budget cycle

9.2 Quality Assurance — PM Accountability (Standard H.2)

The Program Manager owns the QA cycle for the program. The following are non-delegable PM responsibilities:

QA Activity	PM Responsibility	Frequency
Monthly CI incident log review and trend identification	Review all CIs from the month; identify any patterns; brief the PC	Monthly — end of month; documented
Quarterly Critical Incident trend analysis (Appendix C)	Conduct formal analysis; present to ED; identify corrective actions	Quarterly; presented to ED within 10 business days of quarter-end
Bi-annual ISR file audit (Appendix D)	Conduct audit on minimum 25% of active files; assess ISP quality, cultural plans, safety plans, documentation standards	January and July



Annual MCFD Standards Self-Assessment (Appendix E / QA Tool A)	Lead the self-assessment across all Standards A–H; engage PCs in data collection; develop corrective action plan for any gap	Annually in July; completed before July 31
Annual Program Review (MCFD Standard H.2)	Lead or co-lead with ED; compile all QA data; assess program relevance, effectiveness, and outcomes; present to ED and Board	Annually in July; report to MCFD as required by Service Agreement
KPI dashboard — quarterly presentation to ED	Compile all KPIs from monthly reports; present trend data to ED	Quarterly — within 10 business days of quarter-end
Corrective action log management	Maintain the corrective action log; confirm all actions have named owners; report overdue actions to ED	Weekly tracking; monthly reporting
Annual QA binder review and update	Review QA Program Binder (ECC-QA-2026-001); update tools and forms; present changes to ED for approval	Annually in July alongside the annual program review

9.3 MCFD Contract Management — PM Responsibilities

Task	Authority	Frequency
Maintain the Contract Tracking Log (Appendix G) — all reporting deadlines, deliverables, and key dates	Contracted Service Agreement	Ongoing; reviewed monthly
Submit all contracted reports on time — including CI trends, youth outcome data, training compliance, and financial reports	Contracted Service Agreement	Per contract schedule; PM is accountable; never miss a contracted deadline
Notify MCFD Contract Manager proactively of any material service issue, capacity constraint, or compliance risk	Contracted Service Agreement; best practice	As issues arise; before MCFD learns from another source



Participate in all MCFD monitoring visits — facilitate access to all records, staff, and youth	Contracted Service Agreement; CCALA	As scheduled; PM present and engaged
Receive and respond to MCFD monitoring findings within required timelines — develop corrective action plan for any finding	Contracted Service Agreement	Per monitoring report timeline; PM leads response
Initiate Service Agreement renewal process — begin at 6 months before expiry	Contracted Service Agreement	6 months before expiry; ED leads renewal negotiations; PM provides program data
Track all placement authorizations — confirm each youth has current MCFD placement authority	Contracted Service Agreement; CFCSA	At admission; review at each ISP review; flag any expiry



PART 10 — OHS PROGRAM: MANAGEMENT ACCOUNTABILITY

10. OHS PROGRAM — PROGRAM MANAGER RESPONSIBILITIES

Workers Compensation Act s.115–117; OHS Regulation s.3.1–3.3; OHS Program Binder (ECC-OHS-POL-2026-002)

10.1 OHS Legal Accountability (WCA s.115)

The Program Manager holds statutory management accountability for OHS at the program level under section 115 of the Workers Compensation Act. This is not a delegatable obligation — the PM is personally accountable for ensuring the OHS Program is implemented effectively at every facility under their management.

OHS Responsibility	Authority	Frequency
Review all OHS incident investigation reports completed by PCs — co-sign; identify systemic issues	OHS Reg. s.3.4; WCA s.115	Within 48 hours of every investigation; PM co-signs all investigation reports
Submit WorkSafeBC employer reports (Form 7) within 3 business days of any work-related injury requiring medical treatment — confirm with PC	WCA s.150	Within 3 business days; PM confirms submission
Notify WorkSafeBC by phone immediately for serious incidents (hospitalization, fracture, loss of consciousness, amputation)	WCA s.172 — immediate notification required	Immediately by phone: 1-888-621-7233; never delay
Respond in writing to all JHSC recommendations within 21 days	WCA s.128 — management must respond to JHSC in writing within 21 days	Within 21 days of receiving any JHSC recommendation; no exceptions
Conduct or ensure	OHS Reg. s.4.27–4.31	Annually in July; also



the annual workplace violence risk assessment is completed in consultation with the JHSC		following any serious violent incident
Review and co-sign all monthly OHS inspection reports	OHS Reg. s.3.5	Within 5 business days of receipt; PM signs off
Participate in JHSC meetings — PM or designate attends as the management co-chair or management representative	WCA s.125 — management must participate in JHSC	Monthly; PM participates in at least quarterly meetings personally
Respond to all worker right-to-refuse situations promptly — within the OHS Reg. s.3.12 process	OHS Reg. s.3.12; WCA s.10	Immediately on receipt; PM leads the management response; do not retaliate against the refusing worker
Respond to all WorkSafeBC inspection findings and orders within the required timelines — notify ED immediately on receipt of any order	WCA s.185–196	Per order timeline; PM leads response; ED notified immediately of any formal order or stop-work
Ensure all new workers receive site-specific OHS orientation before working independently — verify with PCs	OHS Reg. s.3.22–3.23	At every hire; PM confirms at 30-day check-in
Champion visible safety leadership — raise safety at team meetings, in supervision, and in site visits	WCA s.115 — management leadership	Every interaction; PM models safety culture
Review OHS program performance	WCA s.115; OHS Reg.	Annually; as part of the annual program review



annually and present to ED — include: incidents, near-misses, training compliance, JHSC recommendations, WorkSafeBC interactions		
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10.2 Return-to-Work — PM Responsibilities

- Within 48 hours of any work-related injury, PM confirms: Form 7 submitted; modified duty options identified; RTW plan initiated
- PM maintains regular supportive contact with any staff member on WorkSafeBC leave — minimum bi-weekly; documents contact
- PM works with HR to develop and implement the Return-to-Work plan; approves modified duties within the program
- PM ensures no staff member is penalized, pressured, or disadvantaged for filing a WorkSafeBC claim
- PM presents all RTW cases to ED at the monthly operations update



PART 11 — MANDATORY TRAINING MATRIX: PM AND ALL DIRECT-CARE STAFF

11. MANDATORY TRAINING — COMPLETE REQUIREMENTS

MCFD Standard G.3; OHS Regulation s.3.22–3.24; HR Policy Binder s.5; Residential Care Regulation; Contracted Service Agreement

The Program Manager is accountable for training compliance across the entire program. All training must be tracked on the Training Compliance Matrix, updated monthly. Staff with lapsed mandatory training are immediately removed from duties requiring that training.

11.1 Training Matrix — All Direct-Care Staff and Program Coordinators

TRAINING	MCFD / AUTHORITY	INITIAL DEADLINE	RENEWAL
OHS Orientation + Program Manual orientation	OHS Reg. s.3.22; MCFD Standard G.3	Day 1 — before first independent shift	At each annual update; re-acknowledgement documented
Youth Rights (CFCSA s.70)	MCFD Standard G.3; CFCSA s.70	Before first solo shift — within 7 days	Every 2 years; annual refresher in team meeting
Mandatory Reporting — CFCSA s.14	MCFD Standard G.3; CFCSA s.14	Before first solo shift — within 30 days	Every 2 years; mcf.gov.bc.ca online module
Privacy and FOIPPA	MCFD Standard G.3; FOIPPA	Within 30 days	Annual; oipc.bc.ca module
WHMIS 2015	OHS Reg. Part 5; MCFD G.3	Before first shift	Every 3 years; update on product change
Abuse Recognition and Prevention	MCFD Standard G.3	Within 30 days	Annual
Missing Child/Youth Protocol (MCFD, revised May 2025)	MCFD Standard G.3; MCFD Missing Protocol	Within 30 days	Annual; update when protocol revised
Workplace Violence Prevention	OHS Reg. s.4.27–4.31; MCFD G.3	Before first shift	Annual
First Aid / CPR Level C	OHS Reg. Part 7; MCFD G.3	Within 30 days; before first unsupervised shift	Per certification cycle (annual CPR/AED; 3yr Level C)
OFA Level 2 — designated first aid attendant per shift	OHS Reg. Part 7	Before first duty as designated FAA	Per OFA 2 certification cycle
Non-Violent Crisis Intervention (NVCI) —	MCFD G.3 — safe use of restraint; OHS Reg.	Module 1 before solo shift; full within 90	Annual recertification



full certification		days	
Trauma-Informed Practice foundations	MCFD Standard G.3; best practice	Within 60 days	Every 2 years
Indigenous Cultural Safety and Humility	MCFD Standard G.3; DRIPA	Within 60 days	Annual — San'yas or FNHA approved module
Gender-Affirming Care (MANDATORY — Standard G.3.2(b), amended April 2024)	MCFD G.3.2(b) — MANDATORY since April 2024	Within 60 days	Annual — Trans Care BC or PHSA approved module
LGBTQ2S+ Inclusion and Cultural Humility	MCFD Standard G.3	Within 60 days	Annual
Suicide and Self-Harm Response — ASIST or SafeTALK	MCFD G.3; best practice	Within 60 days	ASIST: biennial; SafeTALK: annual — LivingWorks
Medication Administration (authorized staff only)	MCFD G.3; RCR s.46	Before any medication duty	Annual competency check
Working Alone Safety	OHS Reg. s.4.21–4.24	Before first solo overnight shift	Annual
Driver Safety + annual driver's abstract	OHS Reg. Part 16; Motor Vehicle Act	Before first driving duty	Annual abstract review

11.2 Program Manager — Additional Mandatory Training

TRAINING	AUTHORITY	TIMELINE	RENEWAL
All direct-care staff training above	As above	PM holds all certifications required of the staff they supervise	Per above renewal schedule
Executive-level supervision and leadership	Best practice; MCFD G.4	Within 90 days of assuming PM role	Every 3 years; recommend external executive coaching
Financial management — budget oversight and accountability	Finance Manual; H.1	Within 60 days of assuming PM role	Annual finance update from ED/Finance
MCFD Service Agreement and contract management	Contracted Service Agreement; H.1	Before assuming PM role	When contract is renewed or significantly changed
WorkSafeBC incident investigation — management level	OHS Reg. s.3.4; WCA s.172	Before first investigation as PM	Every 3 years; worksafebc.com advanced module



JHSC co-chair and management representative training	WCA s.125; OHS Reg.	Before serving as management JHSC representative	Every 3 years; WorkSafeBC-recognized
Progressive discipline and workplace investigations	HR Policy s.13–14	Within 60 days of assuming PM role	Every 3 years; HR facilitates
Conflict resolution and grievance response	HR Policy s.17	Within 60 days of assuming PM role	Every 3 years
Indigenous Cultural Safety — Advanced / Leadership level	DRIPA; MCFD G.3; leadership standard	Within 90 days of assuming PM role	Annual
Reflective supervision — facilitated training	MCFD G.4; best practice	Within 60 days of assuming PM role	Annual; recommend external facilitation every 2 years
Media and crisis communications	HR Policy; organizational policy	Within 60 days — even though ED speaks to media; PM must know the protocol	Every 3 years
Licensing and regulatory compliance — residential care	CCALA; RCR; MCFD Standards	Within 60 days of assuming PM role	Annually via MCFD or sector learning events

11.3 Online Training Platforms — Approved Sources

Platform	Modules Relevant to Program Manager Role
WorkSafeBC — worksafebc.com	Incident Investigation (management level); Working Alone; Workplace Violence Prevention; JHSC training; OHS Program development; Young and New Worker orientation
BC Government eLearning Hub — learning.gov.bc.ca	Rights of Children and Youth in Care; FOIPPA for managers; Accessibility; Indigenous reconciliation in public services
MCFD — mcf.gov.bc.ca	Mandatory Reporting; Critical Incident Reporting; Missing Child/Youth Protocol; Child Welfare Policy — full policy manual available online
San'yas — sanyas.ca	Indigenous Cultural Safety Level 1 and 2; Advanced module for leaders and supervisors in social services
FNHA — fnha.ca	Cultural Safety for Health and Social Services Leaders; FNHA Wellness Framework; advancing reconciliation
Trans Care BC / PHSA — phsa.ca	Gender-Affirming Care for Youth (MANDATORY G.3.2(b)); clinical and residential care applications; leadership in affirming environments
LivingWorks — livingworks.net	ASIST (2-day in-person); safeTALK; suicide-safer leadership



Crisis Prevention Institute — crisisprevention.com	NVCI — management-level understanding; restraint reduction program; verbal de-escalation for managers
BC Campus — bccampus.ca	Trauma-Informed Practice for supervisors; open-access community care leadership resources
PFLAG Canada — pflagcanada.ca	LGBTQ2S+ inclusive leadership; organizational culture change for gender-affirming services
BC OIPC — oipc.bc.ca	FOIPPA for decision-makers; privacy breach management; record management best practices
ProQOL — proqol.org	Professional Quality of Life Scale for leaders; understanding and managing secondary trauma in teams
Chartered Professionals in Human Resources BC — cphrbc.ca	HR management in social services; progressive discipline; performance management certification



PART 12 — TASK CALENDARS: DAILY THROUGH ANNUAL

12. PM TASK CALENDAR — ALL FREQUENCIES

12.1 When On-Call — Overnight and Weekend On-Call Responsibilities

☐	Confirm on-call device charged, functional, and forwarded to you at start of on-call period
☐	Remain available by phone at all times during on-call period — respond within 15 minutes to any PC call
☐	For any Category 1 CI reported during on-call: ensure MCFD has been or is being notified; confirm 911 called if needed; confirm staff safety; provide clinical decision support
☐	For any staffing emergency during on-call: arrange emergency coverage; confirm ratio maintained; document the steps taken
☐	For any request to accept an emergency placement during on-call: assess program fit and capacity; confirm or decline; notify ED of any out-of-parameters placement
☐	Brief ED the next morning of any significant on-call event

12.2 Weekly PM Tasks

☐	Review incoming CI Reports from the week — any requiring additional follow-up or MCFD response?
☐	Review weekly staffing schedules from all PCs — confirm ratios; confirm no below-minimum events unaddressed
☐	Review MCFD social worker communications — any outstanding items requiring PM response this week?
☐	Check-in with each PC (informal) — wellbeing; any emerging operational concerns; upcoming critical events
☐	Review corrective action log — any items overdue? Any items requiring PM escalation?
☐	Review approved overtime from the week — any individual approaching fatigue threshold?
☐	Review any outstanding licensing or MCFD reporting deadlines in the next 14 days



12.3 Monthly PM Tasks — by the 20th of Each Month

☐	Receive and review Monthly QA Reports (Appendix D) from all PCs — by the 15th; PM reviews by the 20th
☐	Receive and review Training Compliance Matrix from all PCs — by the 15th; address any lapse immediately
☐	Review all CI Reports from the month — sign off; identify patterns; update corrective action log
☐	Review all OHS inspection reports from PCs — sign off; ensure all Priority 1 deficiencies closed
☐	Conduct individual reflective supervision with every PC — minimum monthly; document in supervision record; include G.4.3(c) gender-affirming care discussion
☐	Review medication error logs and controlled substance count discrepancies — any requiring PM investigation?
☐	Review school attendance data summary from PC reports — systemic issues requiring PM-level action?
☐	Review family contact compliance summary — any program-level barrier requiring PM intervention?
☐	Review youth complaint log — any patterns? Any unresolved complaints requiring PM investigation?
☐	Review financial expenditures for the month — within budget? Any variances requiring ED notification?
☐	Conduct personal site visit to each facility — observe environment, interact with youth and staff, review physical condition
☐	Review JHSC minutes from the month — respond to any recommendations within 21-day window
☐	Update the MCFD Contract Tracking Log — confirm all upcoming deadlines; submit any contracted monthly reports
☐	Review KPI data for the month — update the KPI dashboard for quarterly presentation
☐	Confirm vehicle logs, insurance, and maintenance records are current for all program vehicles
☐	Confirm all CRC records are current — any expiring within 90 days flagged to HR

12.4 Quarterly PM Tasks

☐	Conduct formal Critical Incident Trend Analysis (Appendix C) — present to ED within 10 business days of quarter-end
☐	Present KPI dashboard to ED — trends, concerns, corrective action status
☐	Submit quarterly MCFD program update per Service Agreement reporting schedule
☐	Review ISP review compliance across all youth — all ISPs on 90-day review cycle?
☐	Conduct all-staff meeting — PM present; organizational direction, policy updates, recognition, reflective



	practice
☐	Review corrective action log — close all completed actions; escalate any overdue or unresolvable actions to ED
☐	Conduct or confirm quarterly fire evacuation drill at each facility — review documentation
☐	Confirm annual unannounced fire evacuation drill is scheduled if not yet completed
☐	Review occupancy and waitlist data — notify MCFD Contract Manager of any pattern of under-occupancy
☐	Review training compliance across all PCs — all mandatory training current for all staff?
☐	Review MCFD social worker feedback on service quality — address any concern proactively

12.5 Annual PM Tasks — July Review Cycle

☐	Lead the Annual MCFD Standards Self-Assessment A–H (Appendix E / QA Tool A) — complete by July 31
☐	Lead or co-lead the Annual Program Review per MCFD Standard H.2 — present to ED and Board; submit to MCFD as required
☐	Review and update all written policies and procedures — obtain ED approval; communicate to all staff; obtain signed acknowledgements within 30 days
☐	Conduct bi-annual ISR File Audit (Appendix D) — minimum 25% of active files; July audit
☐	Administer or confirm administration of Youth and Family Feedback Survey (QA Tool F)
☐	Review and confirm Staff Satisfaction Survey has been conducted by HR (QA Tool G)
☐	Review and update the Environmental Risk Assessment for every facility
☐	Conduct or confirm the annual workplace violence risk assessment at each facility in consultation with the JHSC
☐	Conduct or confirm the annual unannounced fire evacuation drill (night or weekend) at each facility
☐	Confirm all vehicle insurance, registration, and driver's abstracts are current
☐	Review all insurance policies — property, vehicle, liability, professional liability; confirm adequate coverage; report to ED
☐	Initiate Service Agreement renewal if the contract expires within 6 months
☐	Confirm licence renewal is on track — application submitted at minimum 60 days before expiry
☐	Conduct annual performance reviews for all PCs — written; both parties sign
☐	Review all job descriptions for program-level positions — update and obtain ED approval for any changes



☐	Prepare and submit the annual budget for ED review — per Finance Manual and organizational budget cycle
☐	Review QA Program Binder — update tools and forms; present changes to ED for approval
☐	Review the annual OHS program performance — present to ED; include for JHSC and Board as appropriate
☐	Confirm all professional development plans are documented and funded for the coming year
☐	Ensure all ProQOL self-assessments have been completed by all staff and reviewed in supervision



APPENDIX A — PROGRAM MANAGER POSITION DESCRIPTION

MCFD Standard G.1 — written job descriptions required; H.1 — organizational chart and management structure

Position Title	Program Manager
Reports To	Executive Director
Direct Reports	Program Coordinators; Senior Youth Care Workers (on programs without a PC)
Classification	Grade F — Senior Management (per HR Policy Binder compensation framework)
Employment Type	Full-Time Permanent
Signing Authority	Per Finance Manual delegated signing authority; does not sign employment offers or contractor agreements

Position Summary

The Program Manager is responsible for the operational leadership of Evergreen Community Care's residential programs — including full accountability for MCFD Standards A through H; all employment relationships for program staff; MCFD and licensing partner relationships; program quality, safety, and compliance outcomes; and organizational financial stewardship at the program level.

Required Qualifications

- Degree in Child and Youth Care, Social Work, or a recognized related field — degree preferred; diploma with substantial management experience considered
- Minimum 5 years of progressive experience in residential child and youth care, including minimum 2 years in a supervisory or management role
- Demonstrated experience managing MCFD contracted services and MCFD Standards compliance
- Current Non-Violent Crisis Intervention (NVCI) certification
- Current First Aid / CPR Level C certification
- Current ASIST or equivalent suicide intervention certification
- Demonstrated experience in HR management including progressive discipline, performance management, and staff development
- Clear Vulnerable Sector Criminal Record Check
- Valid BC Class 5 driver's licence with satisfactory abstract



Core Management Competencies

- Strategic and operational leadership: balances day-to-day operational requirements with program quality and strategic direction
- MCFD Standards expertise: can translate MCFD Standards A–H into operational practice; can identify and address compliance gaps before they become findings
- Trauma-informed clinical leadership: models and enforces trauma-informed practice; provides reflective supervision that shapes clinical culture
- Cultural safety and Indigenous reconciliation: leads a genuinely culturally safe program; builds authentic Nation and DAA relationships; models ongoing cultural humility
- HR management: manages the full HR lifecycle professionally, consistently, and with legal compliance
- Financial accountability: manages program budgets; ensures financial controls; reports accurately
- OHS management: implements the OHS program; supports the JHSC; responds to WorkSafeBC professionally
- Communications: represents Evergreen Community Care professionally to MCFD, licensing, families, and community partners



APPENDIX B — ADMISSION DECISION RECORD

Complete for every placement decision — accepted or declined. Sign and file with the admission record (if accepted) or retain in the referral file (if declined). File retained for minimum 7 years.

Field	
Youth Name (Legal)	
Date of Birth	
Date of Referral	
Referring MCFD Social Worker	
Decision Date	
Decision Made By (Program Manager)	

Assessment Criterion	PM Assessment Notes
Referral authorization: from MCFD or DAA? ☐ Yes ☐ No	
Current licensed capacity available? ☐ Yes ☐ No Current census: ____ Capacity: ____	
Youth's assessed needs within this program's capacity to meet safely? ☐ Yes ☐ Conditional ☐ No	
Safety of current residents assessed? ☐ Safe to proceed ☐ Requires modifications ☐ Not safe to proceed	
CRC current for all staff who will work with this youth? ☐ Yes ☐ Gap identified — action taken:	



Environmental modifications required? ☐ None ☐ Yes — can complete before arrival ☐ Yes — requires delay	
Decision: ☐ ACCEPTED ☐ DECLINED ☐ CONDITIONAL — conditions:	
If declined: MCFD notified in writing by:	

Program Manager Signature / Date	
ED Notification (if out-of-parameters) / Date	



APPENDIX C — QUARTERLY CRITICAL INCIDENT TREND ANALYSIS TEMPLATE

Completed by Program Manager at end of each quarter. Present to Executive Director within 10 business days of quarter-end. Submit summary to MCFD Contract Manager with quarterly report.

Quarter / Year	
Program Manager	
Date Completed	
Presented to ED	

CI Type	This Quarter	Same Quarter Last Year
Category 1 — Total		
Category 2 — Total		
Death or life-threatening event		
Hospitalization		
Abuse / neglect allegation		
Suicidal crisis or attempt		
Self-harm with injury		
Missing youth		
Police contact		
Physical restraint used		
School suspension		
Medication error		



Analysis Area	PM Findings and Notes
Pattern by youth: Are specific youth involved in multiple CIs? Which youth? What is the trend?	
Pattern by time: Are incidents concentrated at specific times of day, days of week, or shift types?	
Pattern by staff: Are specific staff involved in multiple incidents?	
Pattern by type: Is any CI type increasing quarter-over-quarter?	
Restraint trend: Number this quarter vs. same quarter last year. Any prohibited technique? Any threshold youth (3+ in 30 days)?	
Corrective actions: Open actions from prior quarters — are they completed? What is overdue?	
New corrective actions required from this quarter's data:	
Recommendations for program or policy change:	

PM Signature / Date	
ED Review / Date	



APPENDIX D — BI-ANNUAL ISR FILE AUDIT TOOL

Conduct bi-annually (January and July). Audit minimum 25% of active youth files. Findings presented to ED and reported to MCFD at next monitoring visit.

Audit Period	
Number of Active Files	
Files Audited (minimum 25%)	
PM Auditor	
Date Completed	

File Element	Files in Compliance	Files with Gap
Admission documentation complete (all required items)		
Current ISP present; goals measurable; domains complete		
Youth's voice genuinely documented in ISP (not token)		
ISP reviewed within 90-day schedule		
Behaviour Support Plan present (where indicated)		
Cultural Plan present for all Indigenous youth (within 30 days of admission)		
Cultural Plan updated at last ISP review		



Safety plan present and current (where indicated)		
Medical information current (medications, allergies, health alerts)		
MAR complete for current month; no unsigned entries		
Family contact plan documented and being followed		
Education plan and school enrollment current		
Gender-affirming care needs documented in ISP (where applicable)		
Shift logs: 3-log sample — objective, complete, contemporaneous, professional		
CI Reports filed in ISR for all incidents		
Transition plan in place (for youth approaching discharge)		
Discharge summary complete (for discharged youth)		



Findings and Corrective Actions

Finding	Corrective Action Required	Owner / Target Date

PM Signature / Date	
ED Review / Date	



APPENDIX E — ANNUAL MCFD STANDARDS SELF-ASSESSMENT GUIDE

Lead this assessment annually in July. Engage all Program Coordinators in data collection. For each Standard, rate compliance and develop corrective actions for any gap. Submit findings to ED and include in Annual Program Review.

Assessment Period	
PM Lead	
PCs Contributing	
Date Completed	

Standard	Key Questions to Answer	Rating (Met / Partial / Not Met)
Standard A — Program Purpose	Is the Program Description current, accurate, and distributed to all required parties? Are staff operating within the program's stated purpose?	
Standard B — Rights and Complaints	Are all youth rights actively upheld daily? Is the complaint process accessible and functioning? Are all youth rights postings current?	
Standard C — Personal Safety	Is the environmental risk assessment current? Are all CI reports submitted within required timelines? Is restraint use decreasing? Are prohibited practices eliminated?	
Standard D — Quality of Service	Are daily care standards met for all youth? Is gender-affirming care practiced consistently? Are cultural needs being actively supported? Is youth voice shaping care?	
Standard E — Service Planning	Are ISPs developed and reviewed on schedule? Is youth voice genuine in ISPs? Are Cultural Plans complete for all Indigenous youth? Are transitions planned 60 days out?	
Standard F — Shelter and Equipment	Are all facilities safe and in compliance? Are all inspections completed on schedule? Are deficiencies corrected within required timelines? Are fire drills conducted?	
Standard G — Human Resources	Is training compliance at 100%? Is mandatory gender-affirming care training current for all staff (G.3.2(b))? Is supervision occurring monthly for all staff? Are personnel files complete?	



Standard H — Organization and Management	Is the policy manual current? Are all contracts current? Is the annual budget produced? Is the annual program review completed? Are insurance policies adequate?	
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Corrective Action Plan — attach the Corrective Action Log with all gaps identified and assigned to named owners with target dates.

PM Signature / Date	
ED Review and Approval / Date	



APPENDIX F — PERFORMANCE IMPROVEMENT PLAN TEMPLATE

Issue in consultation with HR. ED notification required for all PIPs at management level. Both parties sign. Review at scheduled intervals.

Field	
Employee Name / Position	
PM Issuing PIP	
Date PIP Issued	
HR Consulted — Name / Date	
ED Notified — Name / Date	
PIP Review Dates (schedule all)	1st Review: ____ 2nd Review: ____ Final Review: ____

PIP Section	Detail
1. Performance concern(s) — specific, observable, factual description of the gap between expected and actual performance	
2. Expected standard — what the employee is expected to do; cite the specific policy, job description, or Standard	
3. Improvement goals — specific, measurable objectives for the PIP period; how success will be measured	
4. Support and resources provided — what the organization will provide to help the employee succeed	
5. Review schedule — dates, format, and what will be assessed at each review	



6. Consequences — clearly stated: what happens if the PIP goals are not met within the PIP period	
7. Employee response — employee's perspective on the concerns and their commitment to improvement (optional but encouraged)	

Employee Signature / Date	PM Signature / Date	HR Signature / Date



APPENDIX G — CONTRACT AND PLACEMENT SERVICE AGREEMENT TRACKING LOG

Updated by PM monthly. Review at every monthly operations meeting with ED.

Program / Facility	
MCFD Contract Manager	
Contract Reference Number	
Contract Expiry Date	
Renewal Process Start Date (6 months prior)	

Reporting Obligation	Due Date	Submitted Date	Status
Monthly CI trend summary to MCFD			
Training compliance data (if contracted)			
Quarterly program update to MCFD			
Annual program review submission			
Financial reporting per Service Agreement			
Youth outcome data submission			
Staff roster update (on any change)			
Other: _____			



Licensing Deadlines

Facility	Licence Expiry	Renewal Submission Due (60 days prior)	Status

PM Signature: _____ Date: _____



APPENDIX H — KEY CONTACTS: MCFD, LICENSING, LEGAL, AND EXECUTIVE

PM maintains these contacts current. Verify at monthly operations meeting.

Contact	Name / Number / Notes
Emergency — Police / Fire / Ambulance	911
WorkSafeBC — Serious Incident (24/7)	1-888-621-7233
MCFD Provincial Centralized Screening (PCS) (24/7)	1-800-663-9122
MCFD Contract Manager	Name: _____ Direct: _____ Email: _____
MCFD Regional Office	Address: _____ Phone: _____
Health Authority Licensing Officer (primary contact)	Name: _____ Direct: _____ Email: _____
Health Authority Licensing Office (main)	Phone: _____ Address: _____
Executive Director	Name: _____ Direct: _____ Cell: _____
HR Manager / Director	Name: _____ Direct: _____
Finance Manager	Name: _____ Direct: _____
Organizational Legal Counsel	Name: _____ Firm: _____ Direct: _____
WorkSafeBC Account Manager	Name: _____ Direct: _____
JHSC Worker Co-Chair	Name: _____ Direct: _____
Advocate for Children and Youth (RCY)	1-800-476-3933
MCFD Ombudsperson	1-800-567-3247
Helpline for Children	1-800-663-9122
Poison Control	1-800-567-8911



WorkSafeBC Claims (24/7)	1-888-967-5377
DAA (primary — name of organization)	Name: _____ Contact: _____ Phone: _____

Version History

Version	Date	Summary
1.0	July 1, 2026	Initial Program Manager Manual; all tasks aligned to MCFD SHSS Standards A–H; includes complete training matrix per Standard G.3 including mandatory gender-affirming care training G.3.2(b) April 2024 amendment; OHS management responsibilities per Workers Compensation Act s.115; HR management responsibilities; MCFD contract management; financial oversight; full QA calendar including quarterly CI trend analysis and bi-annual file audit

Signatory	Signature / Date
Executive Director	Signature: _____ Date: _____
Board Chair (or Delegate)	Signature: _____ Date: _____